



# Patient Interviews at Mental Health Sites (Mental Health) Guideline

## Preamble

### Consumer Participation

The Rockingham Peel Group (RKPG) recognises and fully supports the need for the patient and/or their primary carer to<sup>1</sup>:

- Know and exercise their healthcare rights.
- Be engaged in their healthcare.
- Have access to information about treatment options.
- Participate in treatment decisions.
- Have access to information about agreed treatment plans.

### Scope of Practice

It is the responsibility of each health care provider to ensure all patient care and therapeutic interventions they provide are within their individual scope of practice. Refer to the [RKPG Guidelines to Writing for Clinical Policy](#) for further information about Scope of Practice.

## General

- This document outlines the key actions and considerations to be made when undertaking interviews with mental health patients in the inpatient unit and the community mental health clinics.
- Whilst facilitating patient interviews staff should consider representation and advise from carers and cultural representatives where appropriate.

## Mimidi Park inpatient unit

### Room booking

- The Ward Clerk will maintain a booking system for the interview room and other rooms.
- The Ward Clerk will ensure that regular bookings are scheduled to ensure availability of rooms at scheduled times.
- Staff planning interviews will book the interview room/other room in advance where possible.

### Room allocation

- During times of high demand for interview rooms, the Shift Coordinator will allocate rooms. The results of a risk assessment will be utilised to prioritise use of the interview rooms for patients assessed as posing actual/potential risks.
- Patients assessed as low risk may be interviewed in other suitable locations in the communal areas of Mimidi Park. Staff must be mindful of confidentiality requirements when choosing a location for an informal or formal interview.

| RESPONSIBILITY    |                                                                        |              |          | NATIONAL STANDARDS                                 |
|-------------------|------------------------------------------------------------------------|--------------|----------|----------------------------------------------------|
| First Issued      | Oct 2010                                                               | Revisions    | Jul 2018 |                                                    |
| This version      | Dec 2021                                                               | Revision Due | Aug 2025 |                                                    |
| Responsibility of | Co-Directors<br>Mental Health                                          | Endorsed by  | DCS/DONM |                                                    |
| Title             | Patient interviews at Mental Health Sites<br>(Mental Health) Guideline |              |          |                                                    |
|                   |                                                                        |              |          | National Standards for Mental Health<br>1.11, 1.15 |

### Risk assessment and safety

- A risk assessment must be undertaken prior to moving the patient to a room for an interview.  
All staff attending an interview must be aware of the duress system and how to access it.
- All voluntary or involuntary patients have the right to consult with a non-treating medical practitioner, for the purpose of further opinion in regard to treatment (Mental Health Act [MHA] 2014 s.182) and legal practitioners of their choice under the provisions of the Mental Health Act 2014. See also:
  - \* [RKPG Police Attendance at Mimidi Park \(Mental Health\) Guideline](#)
  - \* [Clinicians' Practice Guide to the Mental Health Act 2014](#)
- A patient may request or refuse, verbally or in writing, an interview with medical or legal practitioners.
- A patient may contact the Mental Health Law Centre (Western Australia) Inc. or an independent legal representative for legal assistance.
- The Nurse Unit Manager (NUM) will discuss with the Consultant Psychiatrist any concerns about the safety implications of any planned interview.
- Any medical or legal practitioner requesting an interview will arrange a suitable time with the patient and the NUM.
- The NUM will ensure that an interview room is booked.
- The NUM will ensure that the relevant details of all visits are recorded in the patient's medical record and daily ward report.
- If the patient is assessed as unfit for an interview, or if "there is a serious risk to the safety of the legal practitioner", the Consultant Psychiatrist can restrict the patient's communication or visits by the legal practitioner (MHA s.262 subsection 3) and should notify the Mental Health Medical Co-Director in writing, stating reasons and an estimate as to when the patient will be well enough to be visited for an interview.

### Legal practitioners

- Prior to a visit with a patient, the legal practitioner will contact the Consultant Psychiatrist responsible for the patient and outline the purpose of the visit.
- Prior to the interview, the Consultant Psychiatrist, in conjunction with the NUM, will advise the legal practitioner of any potential difficulties associated with the patient and will suggest strategies for engagement with the patient.
- The interview will be remotely monitored by nursing staff so as to maintain confidentiality between the legal practitioner and the patient.

### Medical practitioners

- An external medical practitioner may not visit a patient without prior permission from the Consultant Psychiatrist unless the visit is in the role of a relative or friend.
- A Consultant Psychiatrist engaged by the patient or the treating Psychiatrist for the purpose of giving a second opinion is permitted to examine patients.

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## Rockingham Community Mental Health

- Rockingham Community Teams facilitate consumer interviews and assessments in three buildings, Older Adult (Goddard Street), Adult (Ameer Street) and the Catalpa centre.
- Each building has at least one safe interview room with two doorways, and a number of rooms used by clinicians to see consumers with one doorway.
- Older adult has two interview rooms; one with a door opening out to the outside and one with two doors opening into the same space.
- The service is not covered by security guards on site.

### Room booking

- Staff planning interviews will book the interview room in advance where possible through the Acute Treatment Team (ATT) diary system.
- If the interview room is needed for an urgent appointment, then staff will check with the ATT diary and/or staff before the room is used.

### Risk assessment and safety

- A risk assessment must be undertaken prior to inviting the consumer to a room for an interview.
- Consumers with a relevant history of destructive/aggressive behaviour should be assessed with a view to using the safe interview room and/or having additional staff present.
- Consumers who are unknown to the service and are presenting as agitated, hyper-vigilant, or with other concerning behaviours should be considered for assessment in the safe interview room.  
Consumers should be asked if they have any items on them that could present as a risk prior to entering the room; if so, staff should manage this as described below.
- The risk assessment may indicate the need for a security presence to be brought in and this request should be brought to the attention of the Team Leader/Program Manager as soon as practicable.
- All staff attending an interview must be aware of the duress system and how to access it.

### Hazardous items

- If a consumer presents with hazardous items (including but not exclusively knives, syringes, other items carried to be a potential weapon) then staff should decide whether or not any interview will take place.
- The consumer should be asked to hand over to staff any items identified as a hazard, by placing them on a flat surface (table, floor etc.) and stepping a safe distance away from them so that staff can retrieve without placing themselves at risk.
- Illicit substances; in the case of suspected illicit substances, staff should make provision for safe storage or/and disposal in collaboration with the Police and senior member of staff and consumer.
- This action is to be documented and a description of the articles made.

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- Weapons; where an offensive weapon is found, the Consultant or Team Leader should be informed immediately to discuss the potential involvement of the Police.
- If the assessment is to proceed then this will be facilitated in the safe interview room.
- The decision to return the items is to be made with the Team Leader/Program Manager or the most senior person on duty.

#### Use of safe rooms

- Where designed, the consumer and any person accompanying them should sit in the chairs nearest the exit that opens into the public space; and staff nearest the exit that opens into the staff area.
- At the beginning of the interview/assessment the consumer must be reminded that they can leave at any time and to use the door behind them.
- In the event that staff need to exit due to an untoward incident the duress should be activated to summon assistance, staff remove themselves from the area and press the door lock button on the outside of the room to prevent the consumer from following.
- Staff should be aware of and follow 'Emergency Procedures Manual Community Mental Health' (available in hard copy only). Staff should be aware of [API Recognition and De-escalation of Behaviour](#).

### **Peel Community Mental Health**

- Peel Community Teams facilitate consumer interviews and assessments in one building situated in Peel Health Campus.
- The service has two safe interview rooms with two doorways, and a number of rooms used by clinicians to see consumers with one doorway.
- The service is not covered by the security guards on site.

#### Room booking

- Rooms can be booked through the reception staff in each building.
- Staff planning interviews will book the interview room/other room in advance where possible.

#### Risk assessment and safety

- A risk assessment must be undertaken prior to inviting the consumer to a room for an interview. Clear documentation regarding the risk assessment must be recorded in the progress notes.
- Consumers with a relevant history of destructive/aggressive behaviour should be assessed with a view to using a safe interview room and/or having additional staff present. If assessed as potential/actual risk, the use of security standby assistance should be utilised.
- Consumers who are unknown to the service and are presenting as agitated, hyper-vigilant, or with other concerning behaviours should be considered for assessment in the safe interview room along with a security presence.

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- Any consumer presenting under the influence of substances, consideration must be given as to whether the interview is to proceed.
- Consumers should be asked if they have any items on them that could present as a risk prior to entering the room; if so, staff should manage this as described below.
- The risk assessment may indicate the need for a security presence to be brought in and this request should be brought to the attention of the Team Leader/Program Manager as soon as practicable.
- All staff attending an interview must wear duress pendants (if fitted in your location), be aware of the duress systems in your area and how to access it.

#### Hazardous items

- If a consumer presents with hazardous items (including but not exclusively knives, syringes, other items carried to be a potential weapon) then staff should decide whether or not any interview will take place.
- The consumer should be asked to hand over to staff any items identified as a hazard, by placing them on a flat surface (table, floor etc.) and stepping a safe distance away from them so that staff can retrieve without placing themselves at risk.
- Illicit substances; in the case of suspected illicit substances, staff should make provision for safe storage and disposal in collaboration with the Police and senior member of staff and consumer.
- This action is to be documented and a description of the articles made
- Weapons; where an offensive weapon is found, the Consultant or Team Leader should be informed immediately to discuss the potential involvement of the Police.
- If the assessment is to proceed then this will be facilitated in the safe interview room.
- The decision to return the items is to be made with the Team Leader/Program Manager or the most senior person on duty (Co-Director or On-call Consultant/Executive).

#### Use of safe rooms

- The consumer and any person accompanying them should sit in the chairs nearest the exit that opens into the external public space; and staff nearest the exit that opens into the staff area.
- At the beginning of the interview/assessment the consumer must be reminded that they can leave at any time and to use the door behind them.
- In the event that staff need to exit due to an untoward incident the duress should be activated to summon assistance, staff remove themselves from the area and press the door lock button on the outside of the room to prevent the consumer from following.
- Staff should be aware of and follow 'Emergency Procedures Manual Community Mental Health' (available in hard copy only). Staff should be aware of [Aggression Prevention Intervention Program](#).

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### Responding to an incident in a community building

- **Staff should familiarise themselves with the [RKPG Mental Health Response to Duress Alarm Policy](#).**
- On hearing the duress, available clinical staff are to make their way to the activation site.
- The first clinical staff member attending should assume the role of Incident Coordinator and instruct staff accordingly.
- The Incident Coordinator must, based on the information provided, assess if it is safe for the staff to intervene. If there are imminent concerns for safety to staff or others, then the decision to notify the Police must be taken by the Incident Coordinator.
- All responders must proceed cautiously to the area of the incident and follow the instructions of the Incident Coordinator.
- Members must not resume normal duties until stood down by the Incident Coordinator.
- Priority should be given to:
  1. Assuring safety of public and staff.
  2. Instructing staff to support getting anybody at risk to a place of safety.
  3. Containing the incident.
  4. Assessing the risk of further incident.
  5. In the assessment that the incident may continue or that staff feel unable to manage the incident safely Police should be called via **000**.
- Following the conclusion of an incident, a debrief of all present should be facilitated by the Incident Coordinator or senior staff member. Staff may access additional support following the incident via the Employee Assistance Provider:
  - \* Converge 1300 687 327
  - \* Optum 1300 361 008
- Additionally, clinical incident report (Datix), clinical record, Combined Hazard or Incident Report (CHOIR), Psychiatric Services Online Information System (PSOLIS) alert (if appropriate), and alert sent to RKPG Security for entry to SENTRY system should be made.

### **Compliance monitoring**

Compliance with this guideline will be monitored by any clinical incidents via Datix.

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## References

1. Australian Commission on Safety and Quality in Health Care (ACSQH). The National Safety and Quality Health Service Standards Safety and Quality Improvement Guide: Standard 1 Governance for Safety and Quality in Health Service Organisations. 2nd ed. (Version 2) 2021. Available from <https://www.safetyandquality.gov.au/our-work/assessment-to-the-nsqhs-standards/>

## Links

[Aggression Prevention Intervention Program](#)

[API Learning Resource: Lone Worker & Community Home Visiting](#)

[Clinicians' Practice Guide to the Mental Health Act 2014](#)

[RKPG Mental Health Response to Duress Alarm Policy.](#)

[RKPG Police Attendance at Mimidi Park \(Mental Health\) Guideline](#)

## National Standards for Mental Health Services

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